Lung Cancer

Companion Figures

Dr R. Sinharay — Imperial 2025

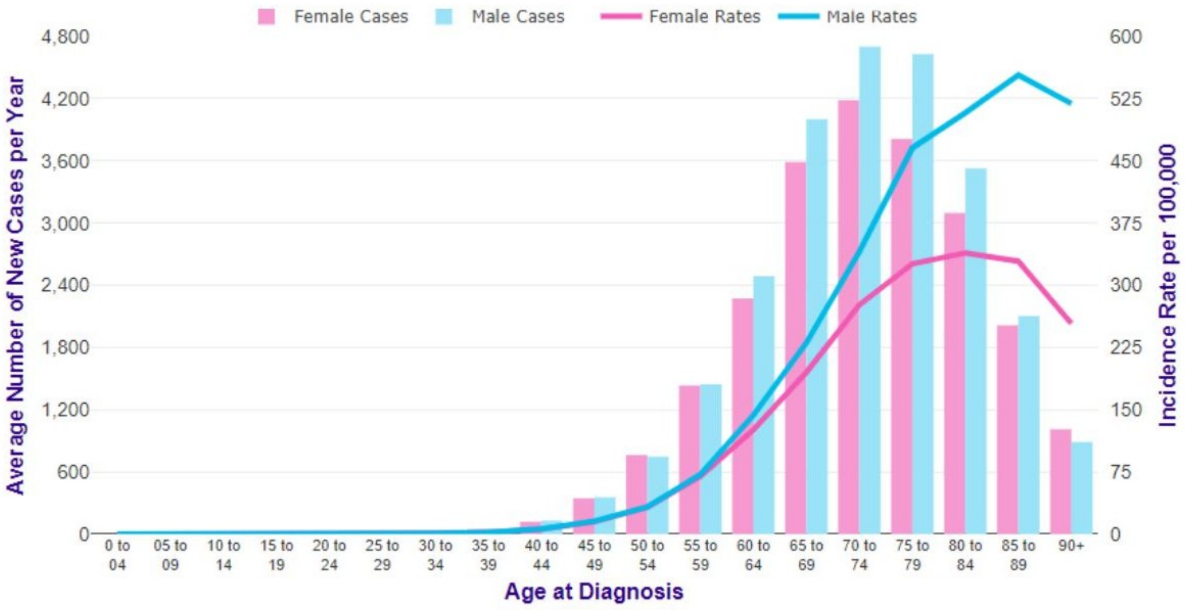
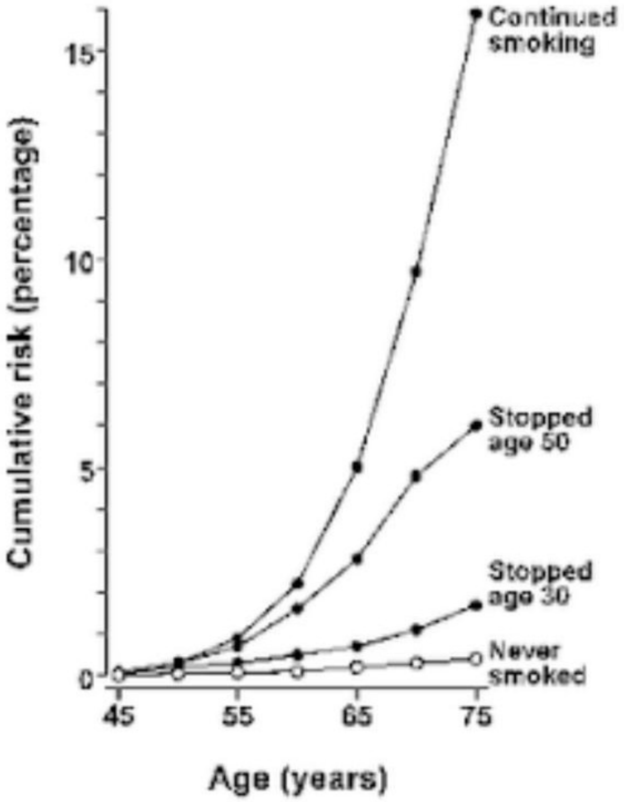


Figure L1.1 — Lung cancer incidence by age and sex (UK)

ure 2.1.1.6. Cumulative lung cancer risk by smoking status and age at quitting king in men in the United Kingdom



ı Peto *et al.* (2000)

Figure L1.2 — Cumulative lung cancer risk by smoking status (Peto 2000)

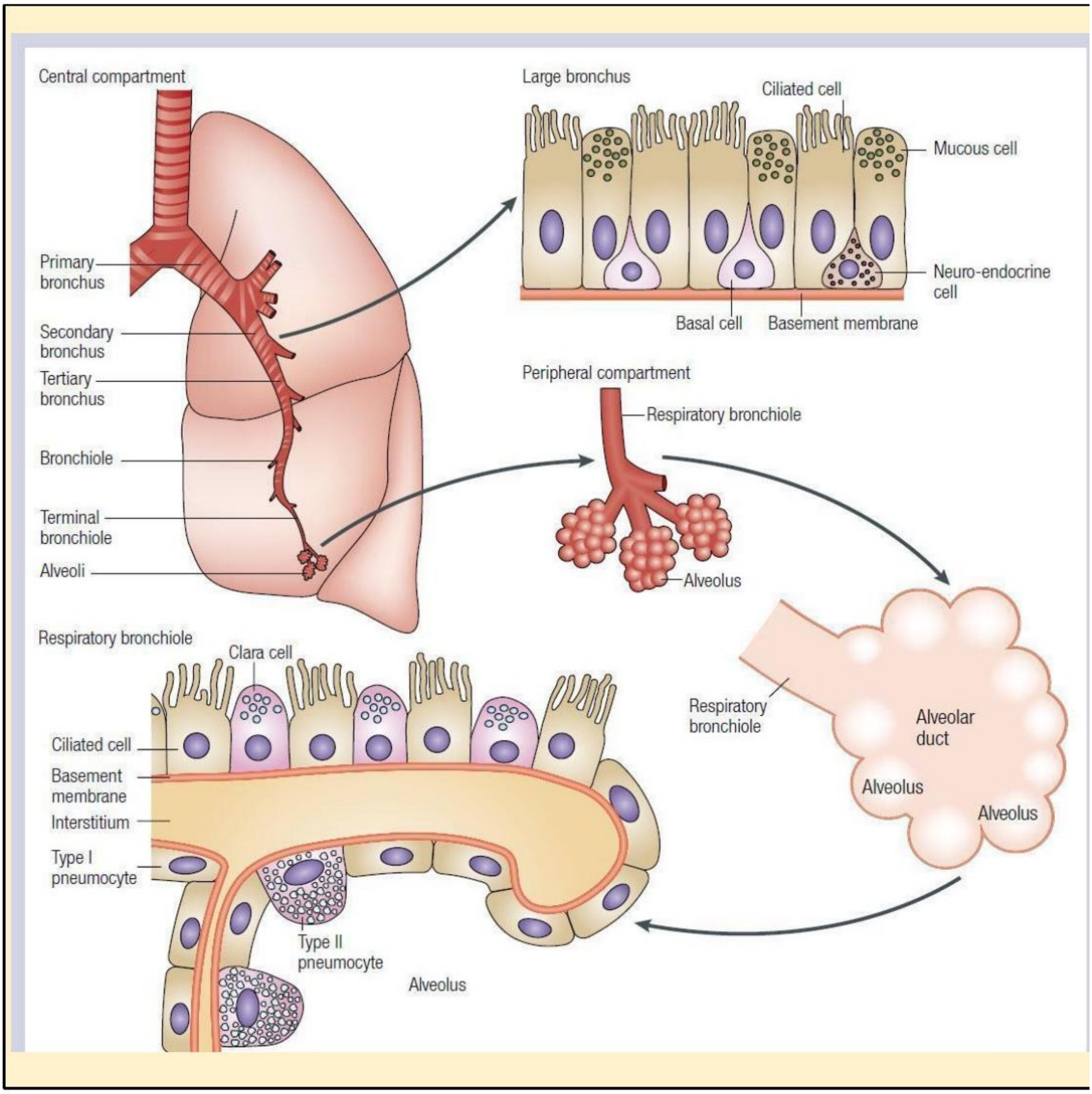


Figure L1.3 — Airway histology: central bronchial vs peripheral alveolar epithelium eatments

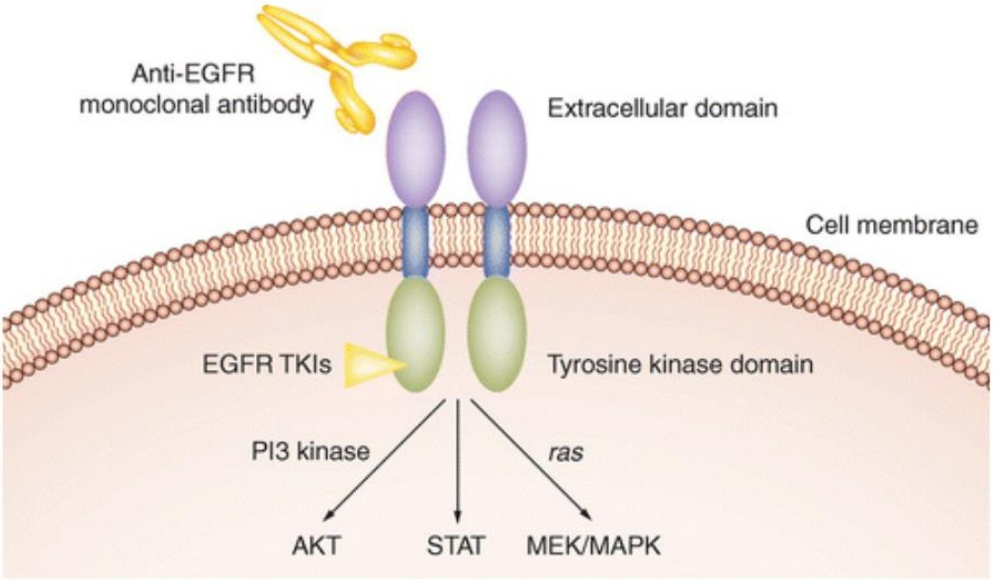


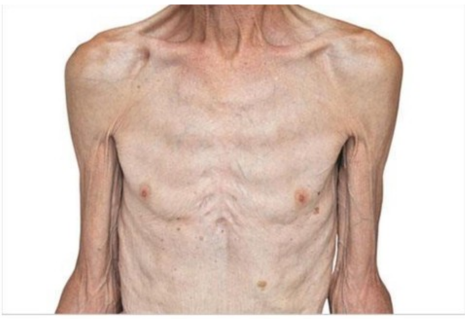
Figure L1.4 — EGFR receptor with TKI and anti-EGFR mAb targets

Signs

clubbing

Source: Usatine RP, Smith MA, Mayeaux EJ, Chumley HS: *The Color Atlas of Family Medicine, Second Edition*: www.accessmedicine.com

cachexia



Superior vena cava obstruction (Pemberton's sign)

Horner's syndrome

Figure L1.5 — Clinical signs: clubbing, cachexia, Horner's syndrome, Pemberton's sign

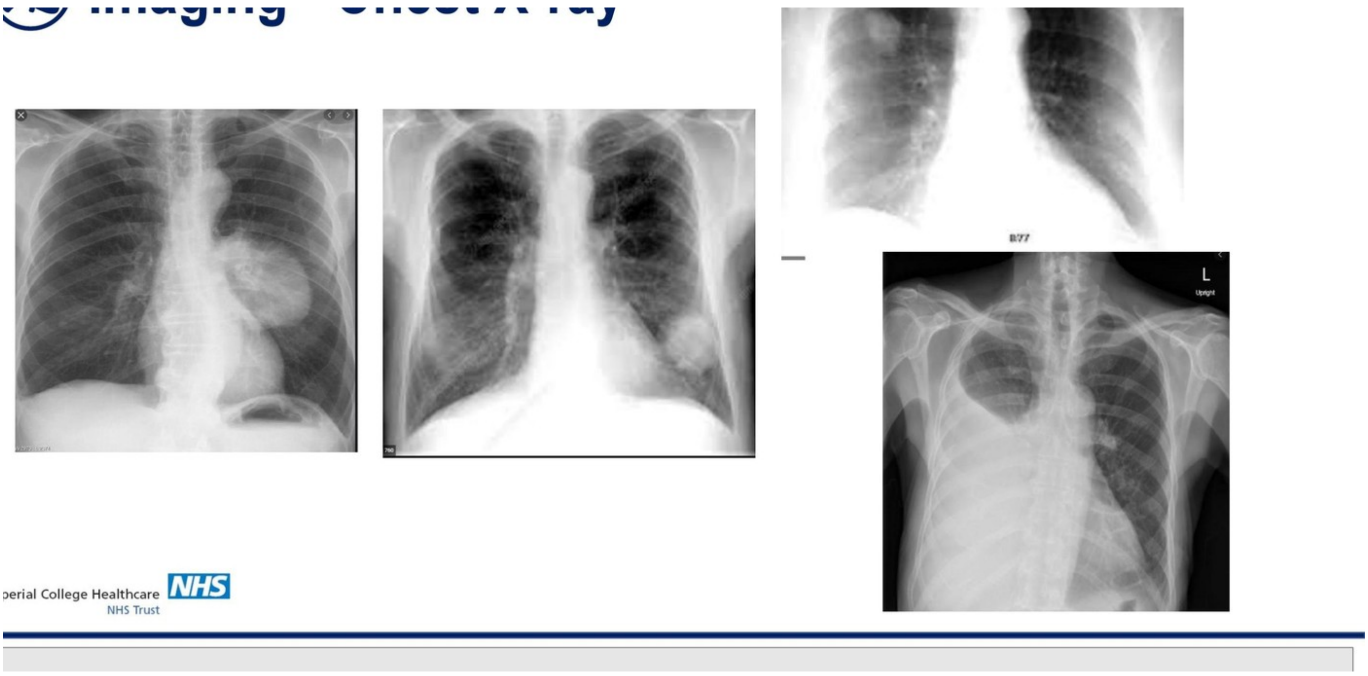


Figure L1.6 — Chest X-rays showing lung cancer (masses and unilateral pleural effusion)

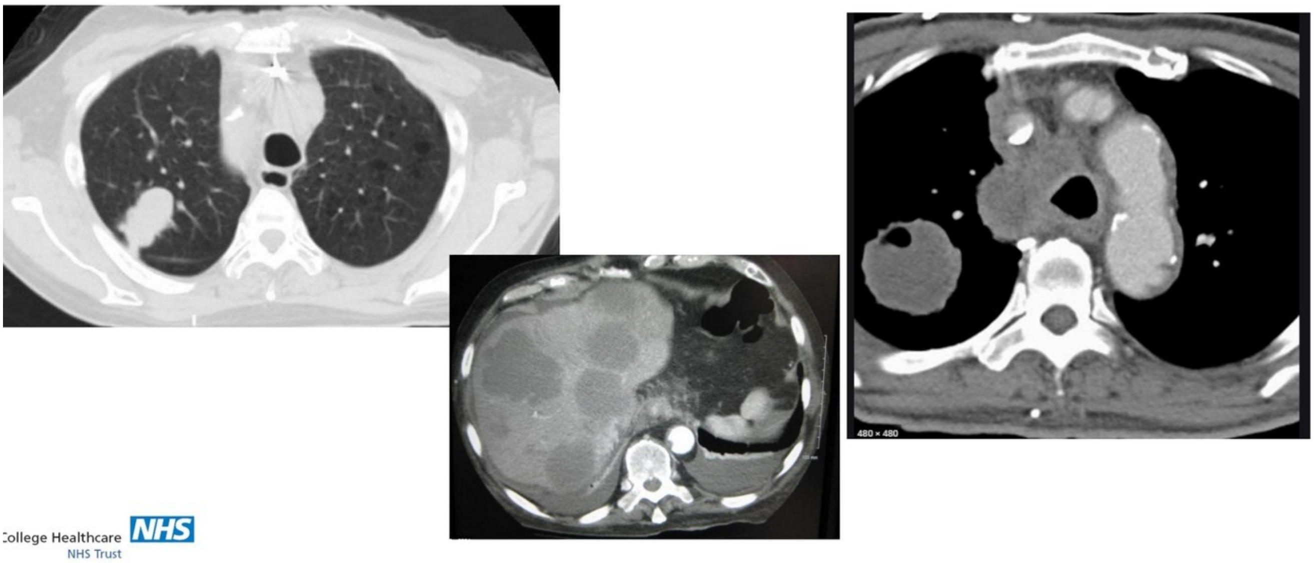
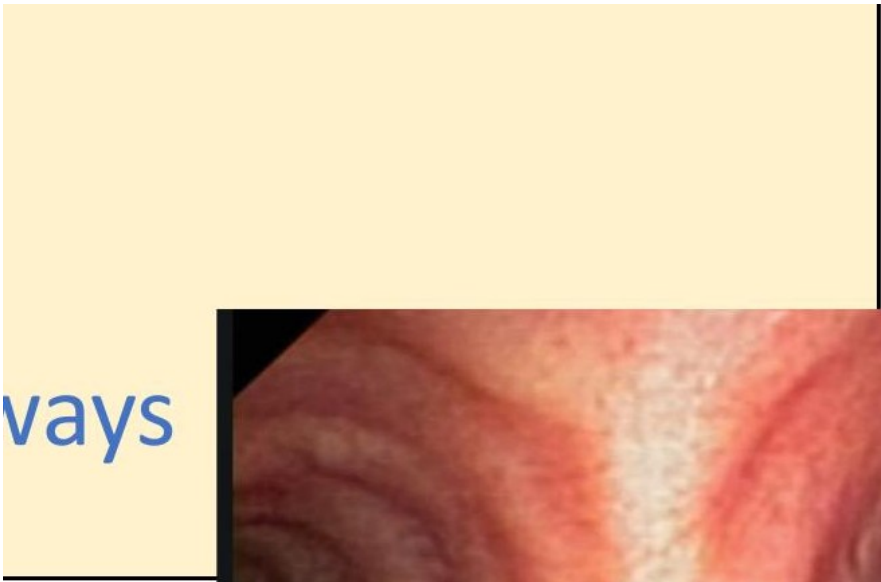


Figure L1.7 — Staging CT: peripheral mass, liver metastasis, mediastinal/bone involvement to exclude occult metastases

Figure L1.8 — PET-CT: FDG-avid right upper lobe primary and mediastinal node



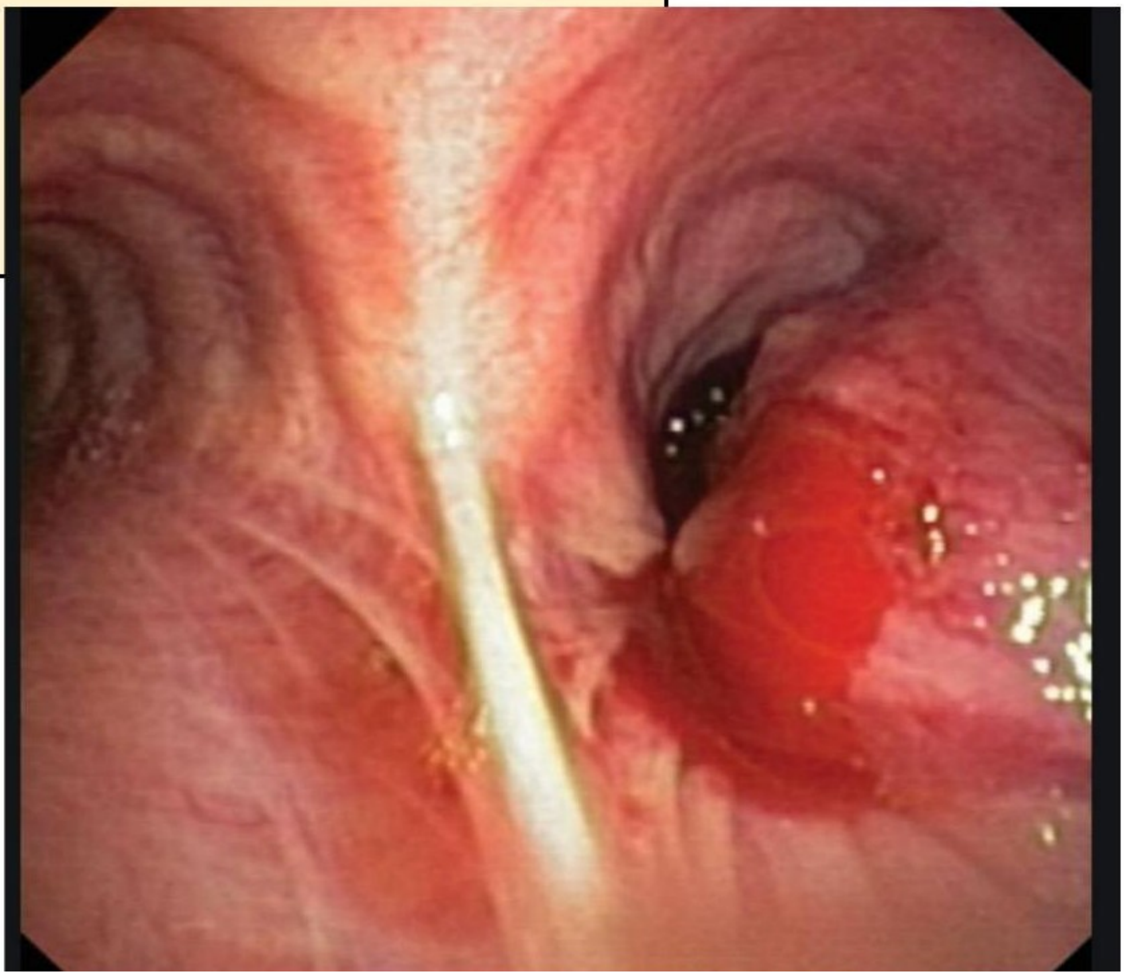


Figure L1.9 — Bronchoscopic view of an endobronchial tumour

liastinum +/- achieve tissue diagnosis

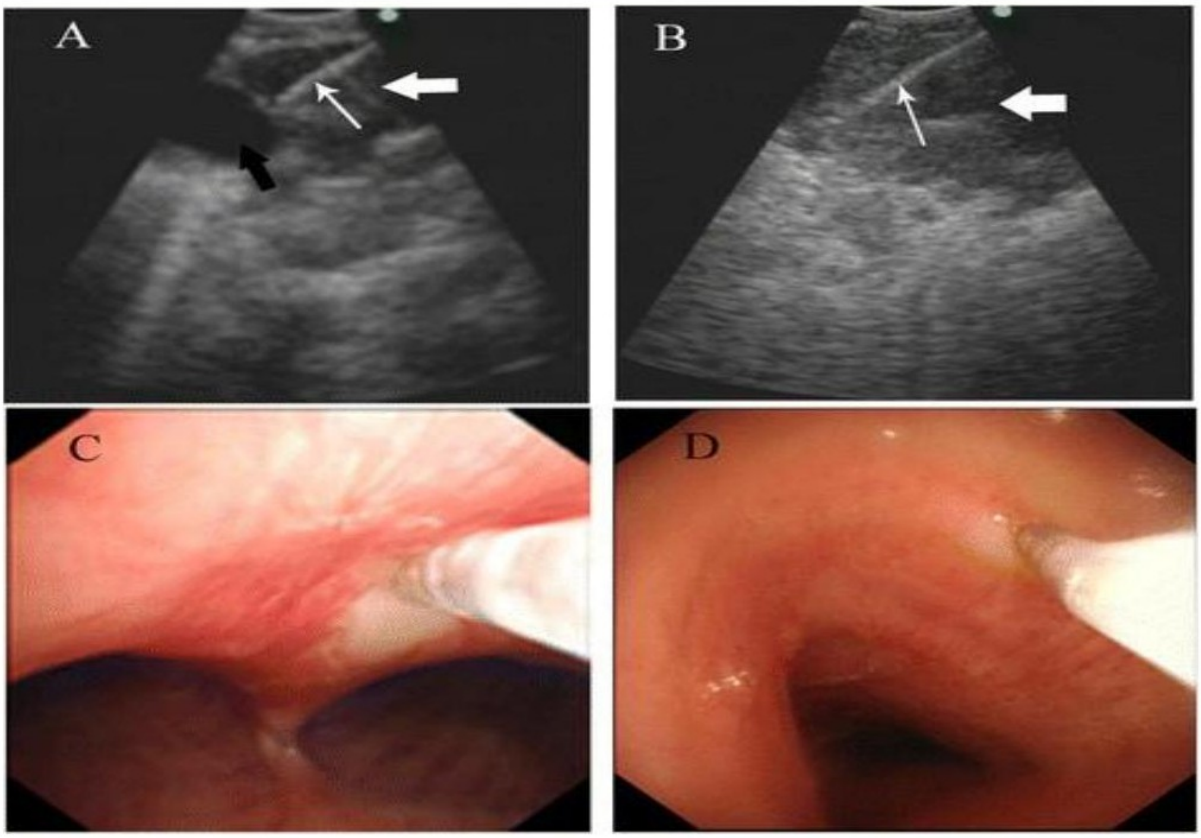


Figure L1.10 — EBUS-TBNA: ultrasound (A, B) and bronchoscopic (C, D) views of mediastinal nodes

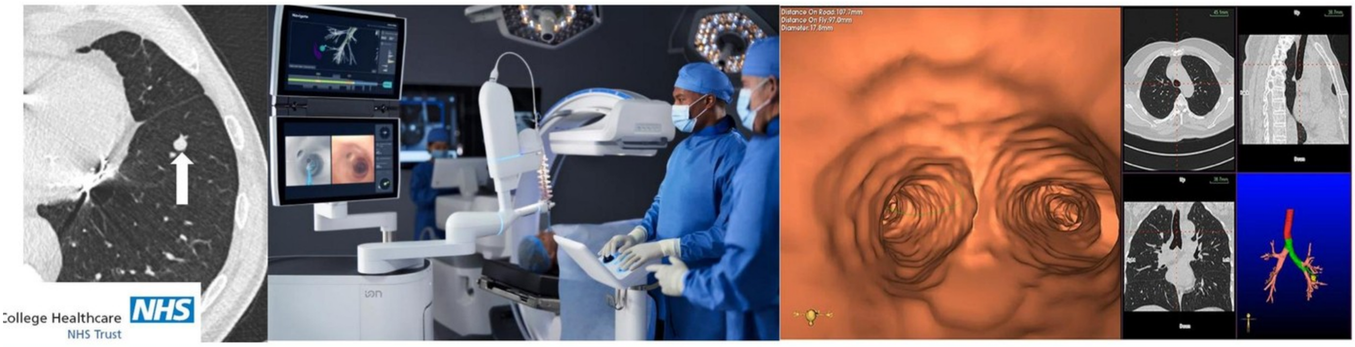


Figure L1.11 — Navigational and robotic bronchoscopy for peripheral nodules

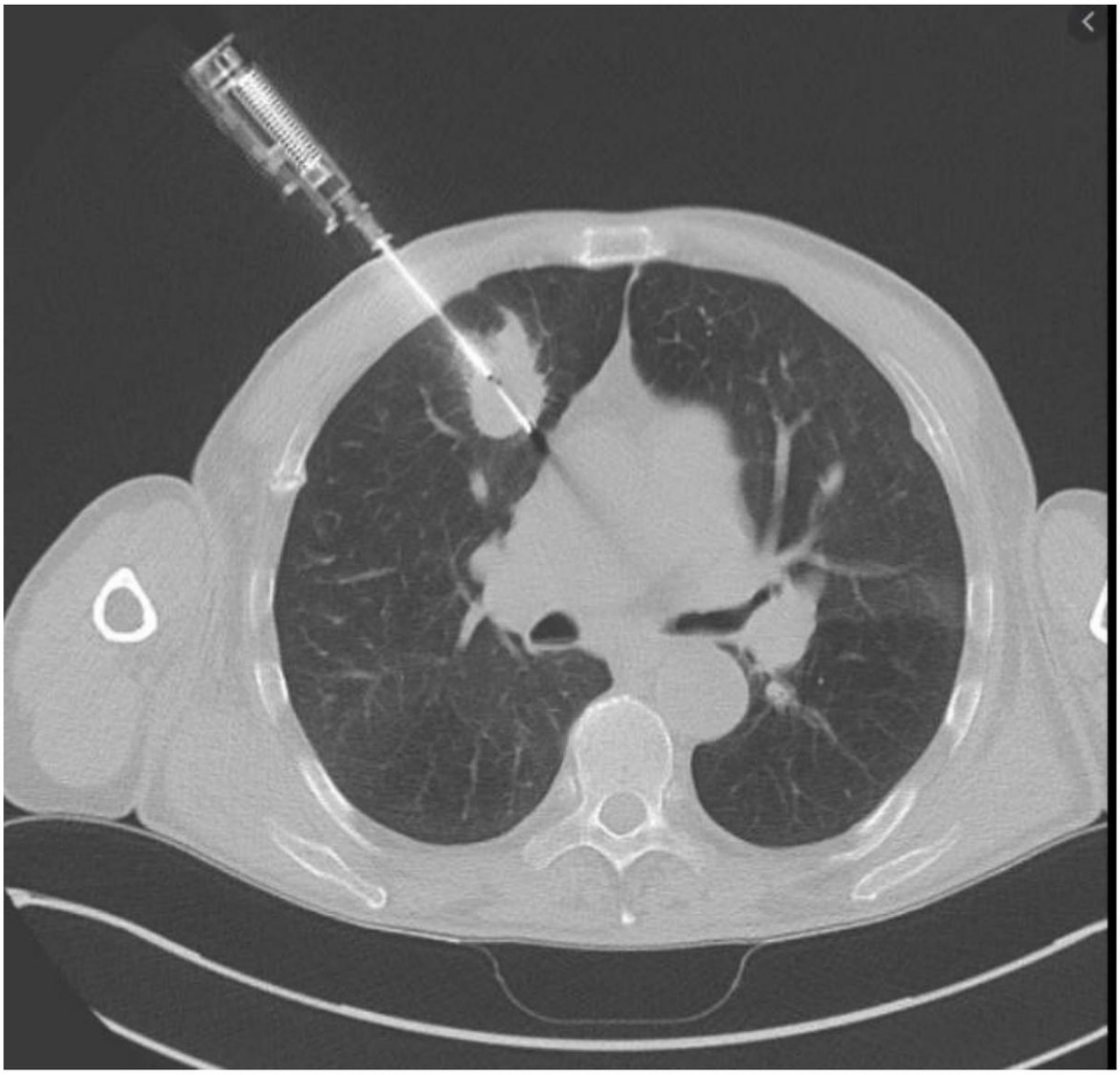


Figure L1.12 — CT-guided percutaneous lung biopsy

TNM 9th edition

Tx	Tumor in sputum/bronchial washings not assessed in imaging/scopy
T0	No evidence of primary tumor
Tis	Carcinoma in situ
T1	• ≤ 3 cm, surround by lung/visceral pleura, or in lobar or peripheral bronchus • T1mi Minimally invasive adenocarcinoma • T1a ≤ 1 cm • T1b >1 cm but ≤ 2 cm • T1c >2 cm but ≤ 3 cm
T2	• T2a > 3cm but ≤ 4cm • or invasion of main bronchus without carina or invasion visceral pleura, transgression fissure, invading adjacent lobe • Atelectasis or obstructive pneumonitis extending to the hilum • T2b >4 cm but ≤ 5 cm
T3	• >5 cm but ≤ 7 cm • or invasion parietal pleura, chest wall, pericardium, phrenic nerve, azygos vein, thoracic nerve roots (T1, T2) or stellate ganglion • or separate nodules in same lobe
T4	• > 7 cm • or invasion mediastinum, thymus, trachea, carina, recurrent laryngeal nerve, vagus nerve, esophagus, diaphragm • or invasion heart, great vessels, intrapericardial pulmonary arteries/veins, supra-aortic arteries, brachiocephalic veins, subclavian vessels, vertebral body, lamina, spinal canal, cervical nerve roots, brachial plexus. • or separate nodules in different ipsilateral lobe

Nx	Regional lymph nodes cannot be assessed
N0	No regional lymph node metastasis
N1	• Ipsilateral peribronchial and/or • Ipsilateral hilar and/or • Intrapulmonary lymph nodes, including involvement by direct extension
N2	• Metastasis in ipsilateral mediastinal /subcarinal nodes • N2a Single N2 station involvement • N2b Multiple N2 station involvement
N3	Contralateral mediastinal/hilar, ipsilateral/contralateral scalene or supraclavicular
M0	No distant metastasis
M1	• Distant metastasis • M1a Pleural or pericardial nodules or malignant pleural or pericardial effusions Separate tumor nodule(s) in contralateral lobe • M1b Single extrathoracic metastasis in a single organ system • M1c1 Multiple extrathoracic metastases in a single organ • M1c2 Multiple extrathoracic metastases in multiple organs

Figure L1.13 — IASLC 9th edition TNM classification for lung cancer

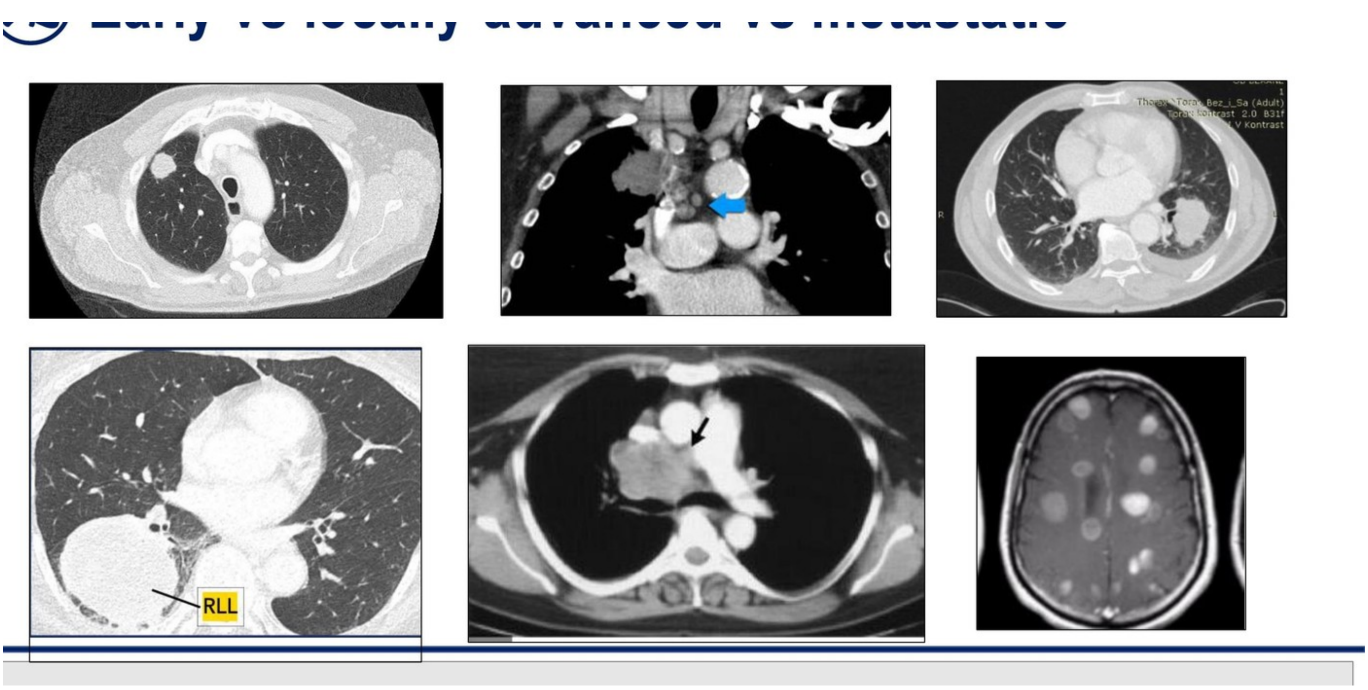


Figure L1.14 — Imaging spectrum: early, locally-advanced, metastatic disease

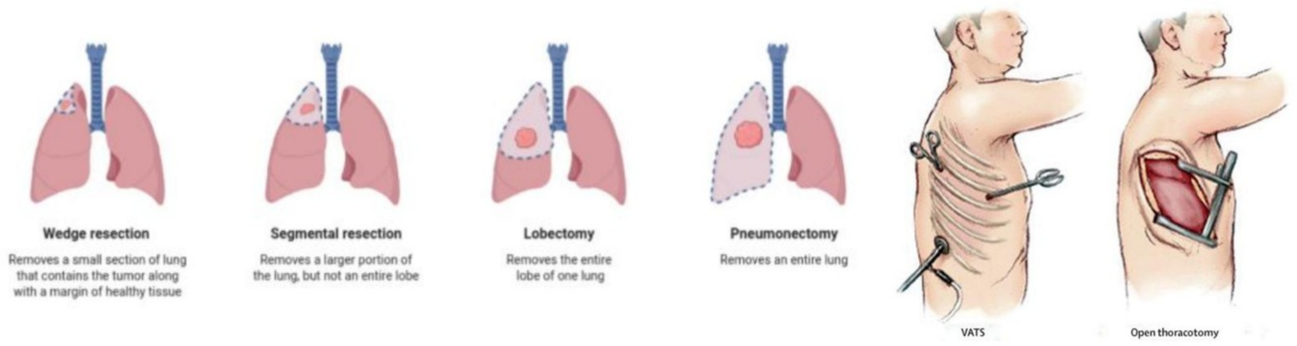
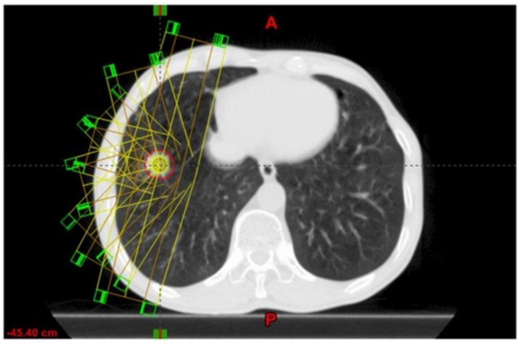
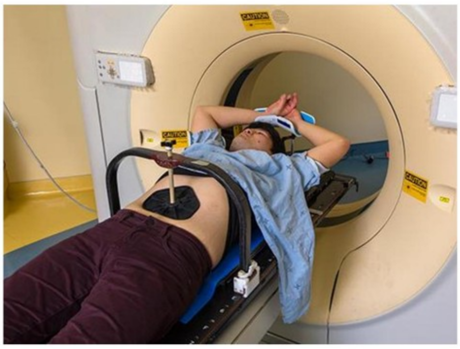


Figure L1.15 — Surgical resection options (wedge, segmental, lobectomy, pneumonectomy) and VATS vs open





-BRS-CVR-9: Respiratory disorders: Describe the clinical features and treatment options of respiratory disorders.

Figure L1.16 — SABR: convergent-beam planning and patient setup stemic treatments: 2a. immunotherapy

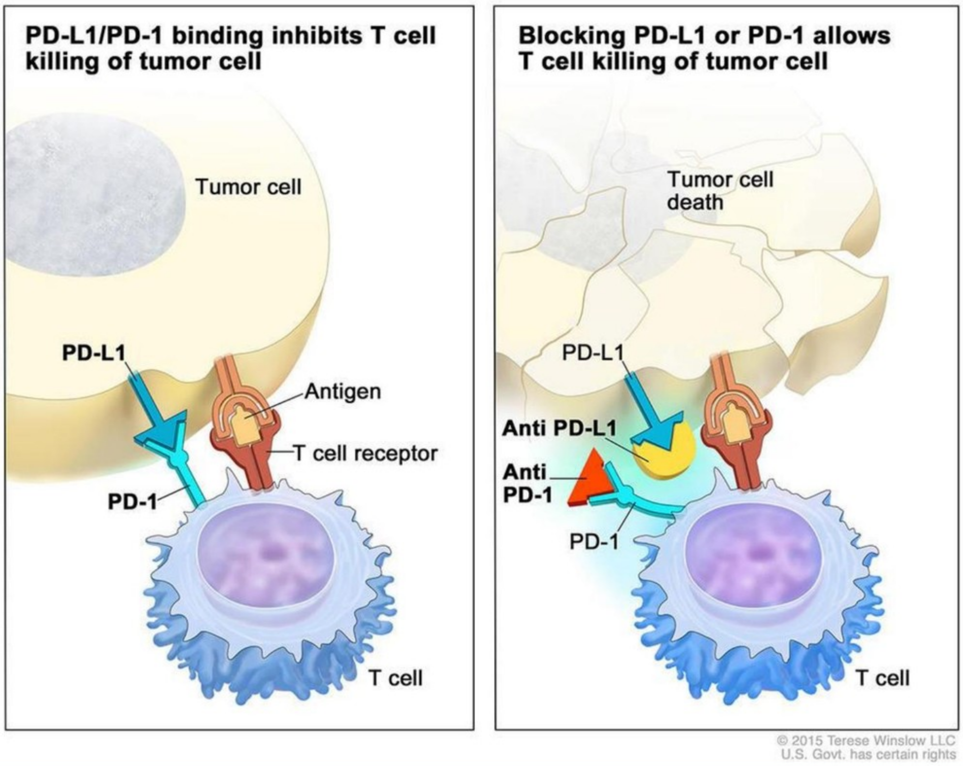


Figure L1.17 — PD-1/PD-L1 checkpoint and immunotherapy mechanism

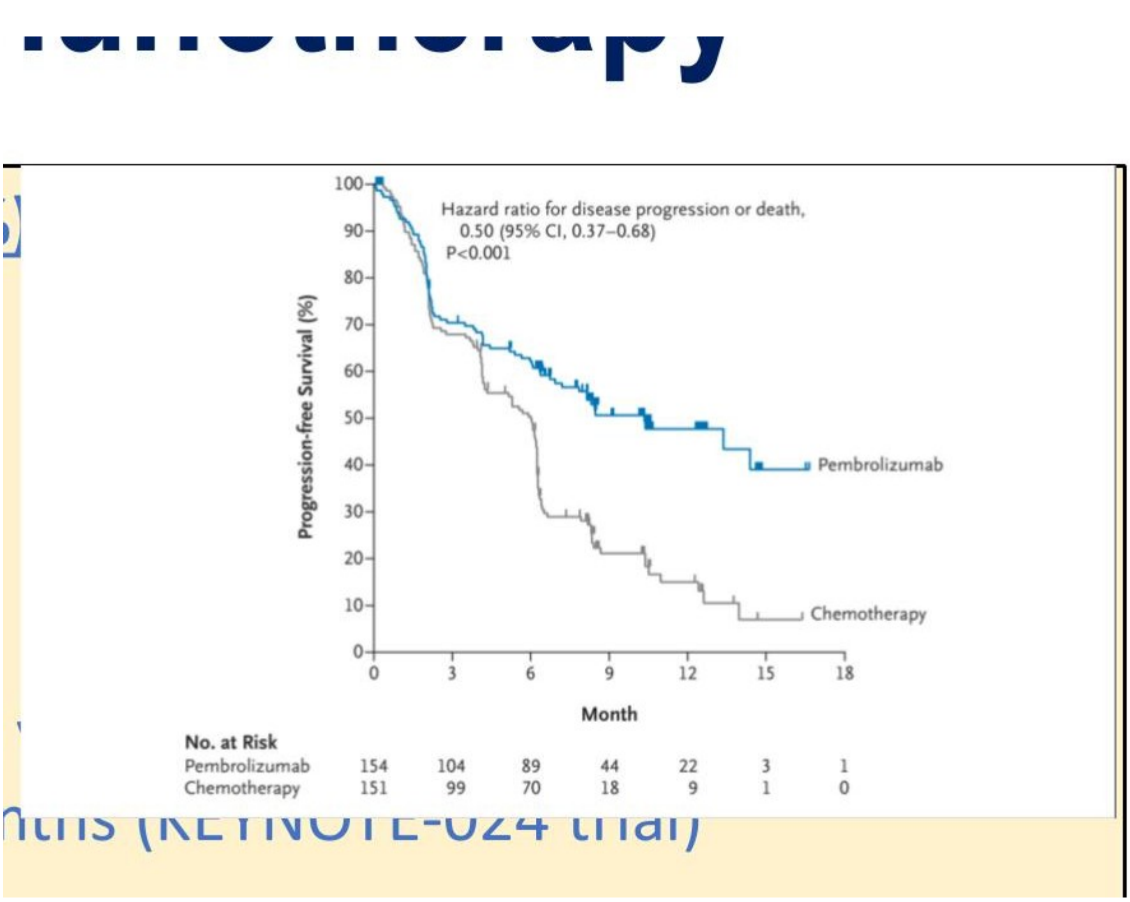


Figure L1.18 — KEYNOTE-024 progression-free survival (pembrolizumab vs chemotherapy)

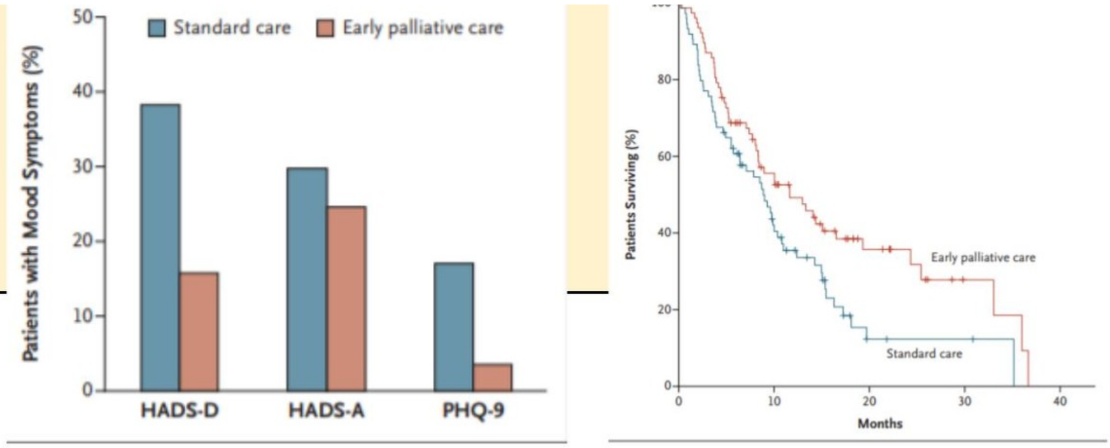


Figure L1.19 — Temel et al. 2010: early palliative care reduces mood symptoms and improves survival

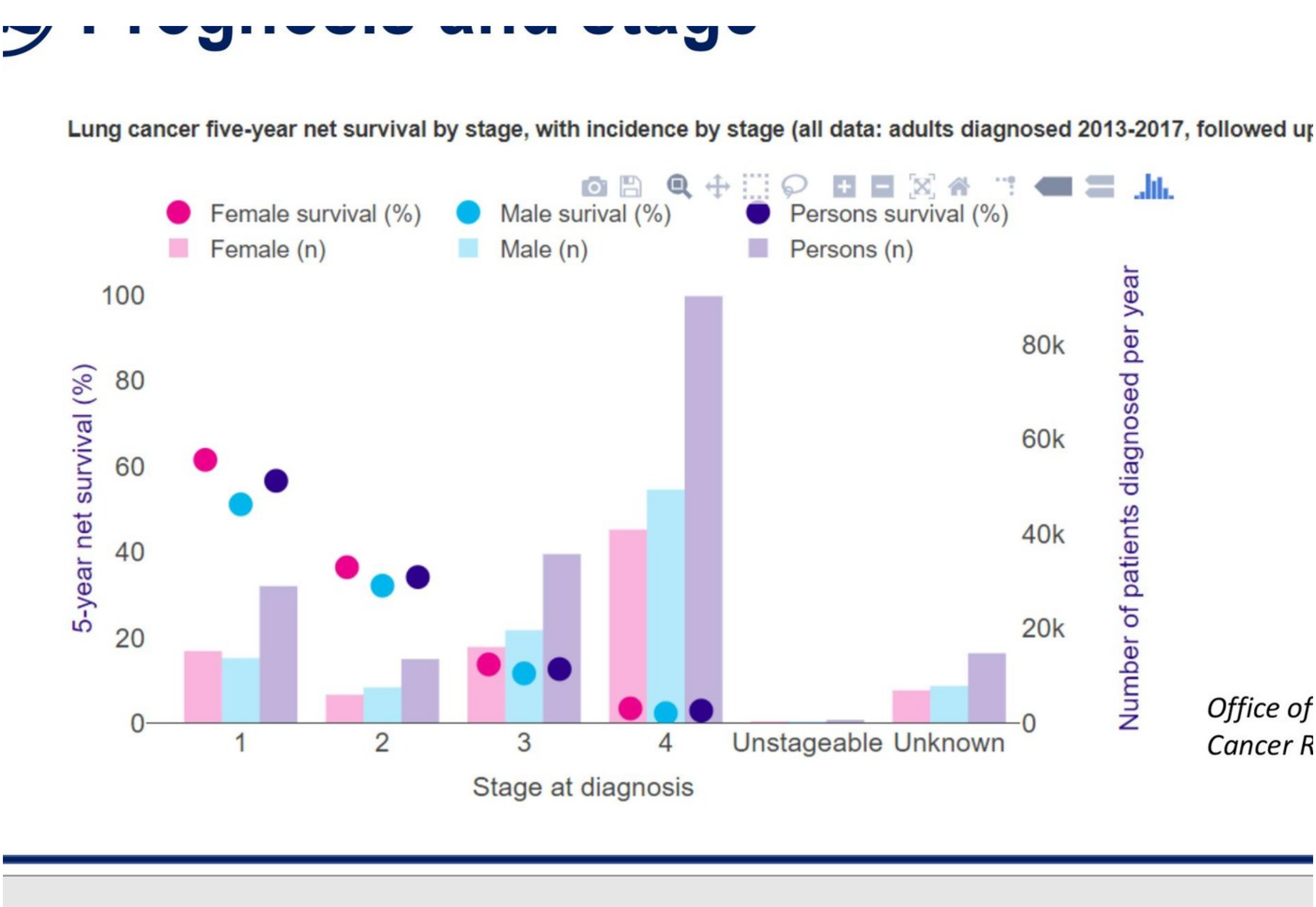


Figure L1.20 — 5-year lung cancer survival by stage with incidence (ONS 2013-17)

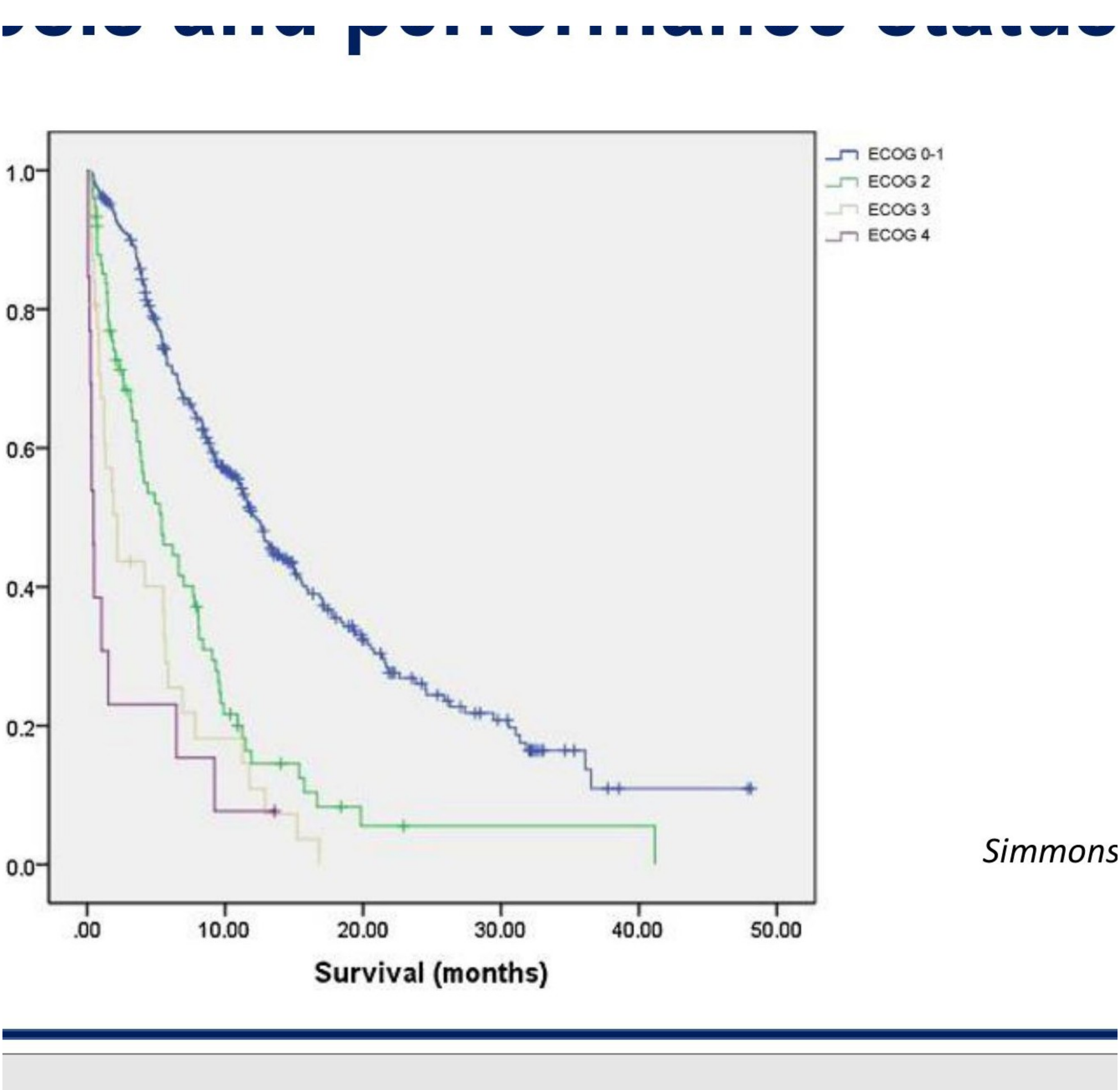


Figure L1.21 — Survival by ECOG performance status (Simmons et al. 2015)